



Clinical Practice Guideline

Management of Nicotine Dependence

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

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These guidelines are intended to guide clinical decision making for clinical staff at North Metropolitan Health Service. They are not strict protocols and do not replace clinical judgement. These guidelines should never be relied on as a substitute for proper assessment of the particular circumstances of each case and the needs of each patient. If staff decide, in exercising their clinical judgment, not to follow the guideline, it is recommended that the reasons are clearly documented in the patient's health care record.

Aim

To guide staff in supporting patients with temporary abstinence or long-term cessation of smoking and vape use (e- cigarettes) by assessing nicotine dependence and prescribing the most appropriate Nicotine Replacement Therapy (NRT). This guideline may also be used to inform and support the assessment, product selection and provision of NRT to staff as part of the Staff NRT Program.

Background

Nicotine dependence is a chronic relapsing medical condition and calls for medical management like any other drug dependency or chronic disease.

Brief advice from a health professional is a major external trigger in prompting someone who smokes to try to quit. Supporting people to quit smoking is one of the most cost-effective and clinically effective healthcare interventions. Best practice care for people who smoke involves a combination of pharmacotherapy (such as NRT) and multi-session behavioural intervention (such as that offered through [Quitline](#)).

Staff are not permitted to assist patients to leave SCGOPHCG premises to smoke or vape. Patients are not permitted to smoke or vape in the company of SCGOPHCG Staff.

Risk

The risk of not following this guideline may result in inadequate management of nicotine dependence.

Definitions

Electronic cigarettes (e-cigarettes)	Battery run devices that heat a liquid (known as e-liquid) to produce an aerosol that a person inhales. E-cigarettes may or may not contain nicotine.
Nicotine vaping products	Designed to be inhaled and contain a nicotine solution (in free-base or salt form). NVPs are available in various forms: e-cigarettes, pods and vape liquids (also known as e-liquids and e-juices) and the nicotine content can vary. E-liquid is usually made up of propylene glycol or glycerine and flavours, however, can contain other ingredients.
Nicotine Replacement Therapy (NRT)	The replacement of nicotine from smoking cigarettes or e-cigarettes with safer alternatives to reduce the symptoms of nicotine withdrawal.
Combination NRT	The provision of fast acting products (such as mouth spray, lozenge, or inhalator) combined with nicotine patches.

1. Patient Assessment

Early detection of withdrawal and preventing the risks associated with withdrawal are the key components for effective and safe patient management. Alcohol, tobacco, and other drugs screening should occur in pre-admission clinic where possible.

Special considerations	
Pregnancy	Consider specialist advice from an Obstetrician about individualised patient management strategies.
Mental Health	Consider the possibility of exacerbation of psychological symptoms.
Older adults	Consider the aspect of longer periods of use and dependence which may complicate treatment and compromise recovery goals.
Aboriginal and culturally and linguistically diverse groups	Consider using interpreters and other communication strategies, as necessary. Consider other cultural aspects that may affect setting, expectations, family involvement, and follow up.

All patients presenting or admitted to SCGOPHCG will be asked if they have used nicotine products in the past 30 days.

Patients will be offered NRT and have the benefits discussed.

All patients will be informed that all Health Department sites are Smoke Free and that a Nicotine Withdrawal Guideline is in place to ensure nicotine dependence is managed during their admission.

Reassurance and early intervention, e.g., combination NRT, will assist both voluntary and involuntary patients to manage this transition.

Patients who use e-cigarettes (vaporisers) also require assessment of their nicotine dependence.

Patients who are identified as high risk may have Cigarettes and lighters / matches confiscated until discharge.

2. 'Ask, Advise, Help' Brief Intervention Model

This model of care is designed to be delivered to all people who smoke regardless of intention to quit, focusing on identifying smoking status and providing care.

([See Appendix 1](#))

Where practicable, all patients at risk of experiencing nicotine withdrawal symptoms are to be managed using the [Quit Victoria](#) 'Ask, Advise, Help' (AAH) Model of Care:

Ask: all patients about smoking status and document this in their medical record.

Advise: all patients who smoke that the service is completely smoke free, they will most likely go into withdrawal and that NRT is available. Advise them to quit in a clear, non-confrontational and personalised way.

Help: by offering referral to behavioural intervention through Quitline (13 7848) and prescribe (or help patients to access) pharmacotherapy, such as nicotine replacement therapy.

A referral can be made to the SCGOPHCG Alcohol and Drug Service (ADS) by:

- Submitting an eReferral
- E-mailing SCGH.ADS@health.wa.gov.au
- Phone 6457 6323

3. Nicotine Dependence

Factors influencing nicotine dependence include:

- inhalation technique,
- number of cigarettes,
- time-to-first cigarette,
- genetic factors,
- vaping device,
- and e-liquid nicotine concentration.

The elimination half-life of nicotine is less than 2 hours, so withdrawal symptoms need to be prevented and managed with the timely and regular provision of NRT.

4. Nicotine Withdrawal

Patients should never be forced to experience nicotine withdrawal symptoms while in hospital.

Patients who are nicotine dependent require effective management with NRT to prevent withdrawal symptoms.

Nicotine withdrawal symptoms are predictable, preventable, and treatable. These symptoms can start within 30 minutes from when a patient last smoked/vaped nicotine.

Nicotine withdrawal in health services is associated with patient anxiety and discomfort, challenging and aggressive behaviours, absconding and poor clinical outcomes and can cause clinically significant distress, particularly within the first 24 hours (when symptoms are most severe).

4.1 Symptoms

There can be variation in the severity and timescale of these symptoms amongst individuals.

The symptoms of nicotine withdrawal include two or more of the following:

- | | |
|---------------------------------|------------------------|
| 1. Anxiety | 6. Increased cough |
| 2. Irritability or restlessness | 7. Dysphoria |
| 3. Reduced concentration | 8. Mouth ulceration |
| 4. Tobacco craving | 9. Insomnia |
| 5. Malaise | 10. Increased appetite |

If the patient is experiencing nicotine withdrawal symptoms, consider the following strategies:

1. Increasing dose of NRT as appropriate
2. Recommending that the patient reduce or halve their caffeine intake (if not already done)
3. Reinforcing the use of appropriate behavioural management strategies e.g., distraction

5. Nicotine Replacement Therapy (NRT)

NRT is most effective when used from admission but can be initiated at any time during the hospital stay.

Combination NRT with behavioural support/counselling is more effective than NRT monotherapy with behavioural/counselling support. [Quitline Referral Form](#)

5.1 Assessing smoking status

Complete the Heaviness of Smoking Index (HIS) ([Appendix 2](#)) to determine the patient's dependence score, used to guide NRT prescribing.

Patients with moderate to high nicotine dependence (as per the **Heaviness of Smoking Index**) are likely to benefit most from combination NRT. It can enhance chances of quitting and is appropriate if the patient continues to experience withdrawal symptoms or has difficulty abstaining from smoking while on the nicotine patch alone.

5.2 Initiating NRT

Prescribe NRT in accordance with State-wide Medicines Formulary (SMF). The SMF is accessible on [Formulary One](#).

Consider the following:

- Contraindications and precautions
- Patients on medications that significantly interact with smoking cessation (refer to Appendix 4)
- Products used during previous attempts to quit.

Patients with high level nicotine dependence may need to wear two 21mg/24hour patches at the same time. Generally, the second patch is worn during daytime hours only (removed overnight) and **must be prescribed by the treating medical officer**.

5.3 Nurse initiated NRT

NRT may be initiated by a nurse/midwife, within their scope of practice. The [SCGOPHCG Nicotine Replacement Therapy Structured Administration and Supply Arrangement \(SASA\)](#) is in place to support nursing staff to autonomously provide NRT when deemed clinically appropriate.

Exclusions from Nurse/ Midwife initiated NRT

Contraindicated	Precaution (medical officer consultation required)
• Non-smokers • Children aged < 12 years • Nicotine hypersensitivity • Phenylketonurics (should not use lozenge) • Menthol hypersensitivity (should not use inhalator) • Buerger's Disease (Thromboangiitis obliterans)	• Persons aged < 18 years • Gastrointestinal disease • Acute myocardial infarction, unstable or worsening angina, severe cardiac arrhythmias • Recent* or planned coronary angioplasty, bypass, graft, or stenting • Peripheral vascular disease • Renal and hepatic impairment • Recent* cerebrovascular accident • Patients in Intensive Care Unit • Mothers of preterm infants • Pregnant women or lactating mothers

*Recent = within the last four weeks

5.4 Pharmacist initiated NRT

NRT may be initiated by a clinical pharmacist, within their scope of practice in accordance with SCGOPHCG [Pharmacist Initiated Medications \(PIMs\)](#) framework and the [Pharmacist Partnered Medication Charting \(PPMC\)](#).

5.5 Nicotine Toxicity

Although rare, the symptoms of nicotine toxicity can include nausea, vomiting, headache, and light headedness.

If the patient is experiencing signs of toxicity, consider the following strategies:

- Reduce the dose of NRT
- Recommend that the patient reduce their caffeine intake
- Remove patch before bedtime, if disrupting daily activities or contributing to insomnia
- Consider Medical Review.

Sleep disturbance is common upon smoking cessation and may not be the result of nicotine toxicity or withdrawal.

6. Specific Patient Groups or Settings

Pregnancy and Breastfeeding

Refer to [WNHS Management of Nicotine Dependence Guideline](#).

Refer to [Women and Newborn Drug and Alcohol Service \(WANDAS\)](#) for specialist service and support in the care of pregnant women with complex alcohol and other drug (AOD) use issues.

Adolescents

Despite limited efficacy data, NRT is safe to use in young people (over 12 years and under 18 years) in conjunction with a counselling program. [Quitline Referral Form](#)

Patients living with mental illness

Refer to [NMHS Mental Health Smoking Care](#).

Elective surgery and preadmission clinics

Patients completing the Patient Health Assessment MR673/MR22.1 may self-identify as a current or recent smoker.

These patients will be offered NRT free of charge and encouraged to stop smoking immediately (or as soon as possible).

All patients attending the Preadmission Clinic will have their current (including recent) smoking/vaping status identified and documented on the following forms,

- Nursing Admission Assessment MR852.2/MR132.2,
- Anaesthetic Record 730.4
- Medication History and Management Plan MR410/804

Smoking cessation advice and early intervention/ referral should be provided at time of consent to surgery.

Patients should be advised to quit for as long as possible prior to surgery (ideally up to 6-8 weeks before surgery). However even brief pre-operative abstinence may have benefits and reduce the risk of complications.

Patients who smoke should be provided with:

- Smoking and Surgery information
- An optional referral to the Alcohol and Drug Service (ADS) by:
 - Submitting an eReferral
 - E-mailing SCGH.ADS@health.wa.gov.au
 - Phone 6457 6323
- [Make Smoking History | Stop Smoking Medications](#)

Interventions should begin 6-8 weeks prior to surgery and address issues that relate to the patient's particular health issues e.g., wound healing, infection rates, post-operative complications, and disease progression. Refer patient to counselling services such as [Quitline referral](#) form

Emergency Department

If the patient requires NRT and is unlikely to be admitted, fast-acting NRT (gum, inhalator, mouth spray) is appropriate first line therapy. For patients likely to be admitted, consider combination NRT as first line therapy.

Managing nicotine cravings in ED patients is an important strategy to manage patient agitation and aggression.

7. Discharge

Encourage patients to continue their attempt to quit or to consider quitting after discharge.

On discharge:

- Document the patient's smoking and vaping status, action taken and the plan for follow up in the discharge summary and communication with the GP.
- Prescribe NRT or other related pharmacotherapy on discharge as appropriate and in accordance with the SMF (see NRT availability on in appendix 3).

It is recommended that if NRT is being used to achieve long term cessation, the treatment regimen should be continued for a minimum of 8 weeks.

- Refer for behavioural interventions (e.g., Quitline) or [SCGOPHCG AoD Service](#).

Compliance, monitoring and evaluation

As per the SCGOPHCG Smoke Free Procedure.

Related Documents

[NMHS Smoke Free Policy](#)

[SCGOPHCG Smoke Free Procedure](#)

[Smoke Free WA Health Policy \(MP 0158/21\)](#)

[DoH Guidelines for the implementation of the Smoke Free Policy](#)

[SCGOPHCG SASA Nicotine Replacement Therapy](#)

[SCGOPHCG Staff Nicotine Replacement Procedure](#)

[SCGOPHCG Medication Management](#)

[Guidelines for the WA Hospital Medication Chart](#)

References

[National Tobacco Strategy 2022-2030](#)

[WA Health Promotion Strategic Framework 2022-2026](#)

[National guidance notes on the elimination of environmental tobacco smoke in the workplace \[NOHSC:3019 \(2003\)\]](#)

[Christensen, D. 6.11 Tolerance, Dependence, and Withdrawal. In Scollo, MM and Winstanley, MH \[editors\]. Tobacco in Australia: Facts and issues. Melbourne: Cancer Council Victoria; 2018. Available](#)
[NMHS Smoke Free Hub](#)
[Quitline Health Professionals](#)

Figure 1: 3-Stepp Brief Advice to Support Patients Who Smoke. Source: Three-step brief advice: Ask, Advise, Help Reproduced with permission from Quit Victoria, 3 step quit smoking brief advice – AAH model, Melbourne, DoH Quitline, 2023.

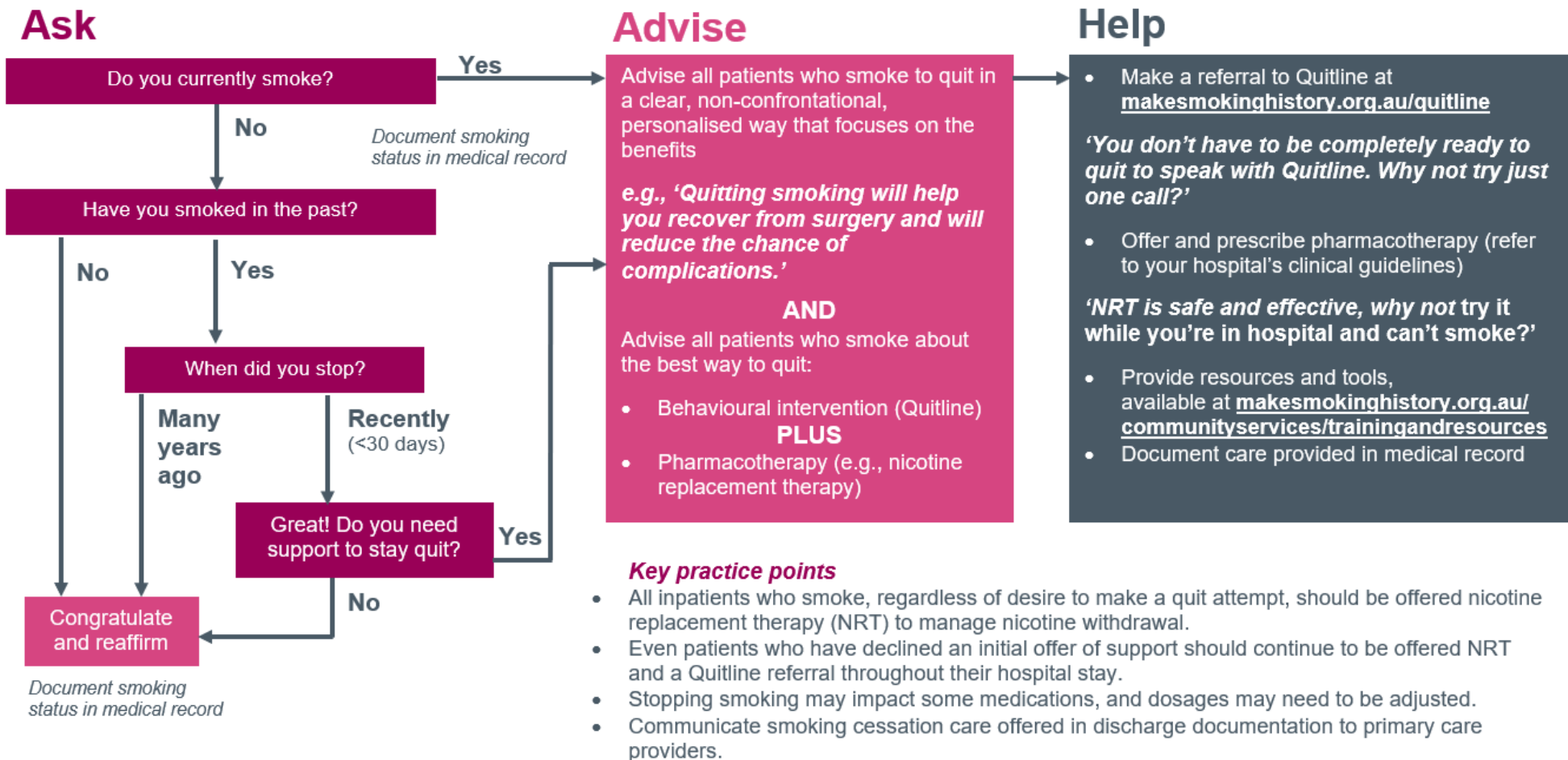
Authorisation & Version Control

Policy Sponsor	Deputy Director, Clinical Services				
Policy Contact	Chair, Smoke Free Working Group / NMHS Health Promotion Officer				
Last Reviewed:		Last Amended:		Review Due:	02/05/2027
Version:		Reference #	CPG037	Aboriginal Health ISD	
Approved by:	Deputy Director, Clinical Services			Date:	02/05/2024
Endorsed by:	SCGOPHCG Policy and Guideline Committee			Date:	
Applicable Standards	NSQHS Standards (Version 2): 				
Printed or personally saved electronic copies of this document are considered uncontrolled					
Document Version Control					
Date	Version	Author	Notes		
02/05/2024	1	Smoke Free Working Group	First version		

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

Appendix 1: 'Ask, Advise, Help' - Brief Intervention Model

3-Step Brief Advice to support patients who smoke



Appendix 2: Offering NRT – cigarette or dual users

Cigarette or dual users

ASK: Assess smoking status by asking the if the patient smokes cigarettes.

If YES, complete the HSI

Heaviness of Smoking Index (HSI)

Q1. How soon after waking do you have your first cigarette?	Within 5 min	☐ 3
	5-30 min	☐ 2
	31-60 min	☐ 1
	60+ min	☐ 0
Q2. How many cigarettes do you smoke each day?"	10 or less	☐ 0
	11 – 20	☐ 1
	21 – 30	☐ 2
	31 or more	☐ 3
TOTAL SCORE		=
DEPENDENCE SCORE LOW = 0-2 MODERATE = 3-4 HIGH = 5-6		

ADVISE: all patients who smoke that the service is completely smoke free (including e-cigarettes), they will most likely go into withdrawal and that NRT is available. Advise them to quit in a clear, non-confrontational and personalised way

HELP:

- by prescribing (or helping patients to access) combination NRT (**nicotine patch and faster-acting NRT e.g., gum, lozenge, inhalator, mouth spray**) based on HSI Score.
- By offering referral to behavioural intervention through Quitline (13 7848).

LOW: HSI score 0-2 or mild cravings with previous quit attempts

NRT may not be required.

Offer **if needed:**

Nicotine 2mg chewing gum

1 piece of gum to be chewed as directed PRN up to every 1-2 hours (Max. 12 pieces in 24 hours)
(Avoid using >1 piece/hour)

OR

Nicotine 2mg lozenges

1 lozenge to be used as directed PRN up to every 1-2 hours (Max. 15 lozenges in 24 hours)

OR

Nicotine 15mg inhalator

The contents of one cartridge to be inhaled PRN (Max. of 6 cartridges in 24 hours)

OR

Nicotine 1mg mouth spray

Use 1 spray PRN up to every 30-60 minutes (Max. of 64 sprays in 24 hours)

MODERATE: HSI score 3-4 or significant cravings with previous quit attempts

Combination NRT is needed.

Nicotine patch 21mg/24 hour

PLUS

Nicotine 2mg chewing gum

1 piece of gum to be chewed as directed PRN up to every 1-2 hours (Max. 12 pieces in 24 hours)
(Avoid using >1 piece/hour)

OR

Nicotine 2mg lozenges

1 lozenge to be used as directed PRN up to every 1-2 hours (Max. 15 lozenges in 24 hours)

OR

Nicotine 15mg inhalator

The contents of one cartridge to be inhaled PRN (Max. of 6 cartridges in 24 hours)

OR

Nicotine 1mg mouth spray

Use 1 spray PRN up to every 30-60 minutes

HIGH HSI score 5-6 or significant cravings with previous quit attempts

Combination NRT is needed.

Nicotine patch 21mg/24 hour

PLUS

Nicotine 4mg chewing gum

1 piece of gum to be chewed as directed PRN up to every 1-2 hours (Max. 10 pieces in 24 hours)
(Avoid using >1 piece/hour)

OR

Nicotine 4mg lozenges

1 lozenge to be used as directed PRN up to every 1-2 hours (Max. 15 lozenges in 24 hours)

OR

Nicotine 15mg inhalator

The contents of one cartridge to be inhaled PRN (Max. of 6 cartridges in 24 hours)

OR

Nicotine 1mg mouth spray

Use 1-2 sprays PRN up to every 30-60 minutes

Ref: QUIT Health Services Clinical Guideline and Pathway Template May 2021

NB: NRT available may vary at NMHS sites. Check with relevant Pharmacy Department.

The Heaviness of smoking index (HSI) is a validated tool to assess level of nicotine dependency from smoking only (combustible tobacco). There is currently no validated tool to assess nicotine dependency from nicotine vaping products (NVPs).

Appendix 3: Offering NRT – nicotine vaping product (e-cigarette) users

Nicotine vaping products (e-cigarette) users

Precaution: this prescribing guidance is for current use of Nicotine Vaping Products only

ASK: Time to first vape

ADVISE: all patients who smoke that the service is completely smoke free (including e-cigarettes), they will most likely go into withdrawal and that NRT is available. Advise them to quit in a clear, non-confrontational and personalised way

HELP: By prescribing (or helping patients to access) NRT based on 'time to first vape'.
By offering referral to behavioural intervention through Quitline (13 7848).

Time to first vape >30 minutes

Nicotine 2 mg chewing gum:

1 piece of gum to be chewed as directed PRN up to every 1-2 hours
(Maximum 12 pieces in 24 hours)
(Avoid using >1 piece/hr)

OR

Nicotine 2mg lozenges

1 lozenge to be used as directed PRN up to every 1-2 hrs
(Maximum 15 lozenges in 24 hours)

OR

Nicotine 15mg inhalator

The contents of one cartridge to be inhaled
PRN up to every 2-4 hrs
(Maximum of 6 cartridges in 24 hours)

OR

Nicotine 1mg mouth spray

Use 1-2 sprays PRN up to every 30-60 minutes
(Maximum of 64 sprays in 24 hours)

Time to first vape < 30 minutes

Combination NRT is recommended.

Nicotine patch 21mg/24 hour

PLUS

Nicotine 2mg chewing gum

1 piece of gum to be chewed as directed PRN up to every 1-2 hours (Max. 12 pieces in 24 hours)
(Avoid using >1 piece/hour)

OR

Nicotine 2mg lozenges

1 lozenge to be used as directed PRN up to every 1-2 hours
(Max. 15 lozenges in 24 hours)

OR

Nicotine 15mg inhalator

The contents of one cartridge to be inhaled PRN (Max. of 6 cartridges in 24 hours)

OR

Nicotine 1mg mouth spray

Use 1-2 sprays PRN up to every 30-60 minutes
(Maximum of 64 sprays in 24 hours)

Notes:

- Consider commencing at higher dosages (including combination NRT) if the patient has experienced severe cravings when previously abstaining.
- Nicotine dependence from the use of NVPs is influenced by nicotine concentration, inhalation technique and vaping device characteristics. The dose of NRT can be increased if the patient has inadequate relief of nicotine withdrawal symptoms (e.g., urges to smoke/vape, irritability, and restlessness).
- 'Dual users' (patients who concurrently smoke and vape) can be initiated on NRT as per the patient's HSI score. For prescribing guidance.
- Check the patient's current medication regimen for any potential drug interactions.

Ref: Alfred Health Clinical Management of Nicotine Dependency Among Patients 2020, Bittoun, R. (2021). Managing vaping cessation: A monograph for counselling adult and adolescent vapers.

NB: NRT available may vary at NMHS sites. Check with relevant Pharmacy Department

Currently, there is no validated assessment tool to assess nicotine dependency from the use of NVPs. Regular review is recommended to adjust NRT as needed.

Appendix 4: NRT Products

Other Prescription Pharmacotherapies			
Product	Time to peak	Use	Administration
 Gum	SHORT ACTING 3-5 minutes	1 piece gum/hour, if required	- Place gum in mouth and chew slowly until a 'tingly sensation' or 'bitter taste' appears. 'Park' the gum either under the tongue or between your gum and cheek until the tingling stops. - Keep repeating the 'chewing and parking' of the gum for up to 30 minutes.
 Inhalator	SHORT ACTING 30-60 minutes	Regular use or as required	- Insert cartridge and close device to puncture - The patient should self-titrate dose according to symptoms with short and shallow inhalations - Do not use while eating or drinking
 Lozenges	SHORT ACTING 20-30 minutes	Regular use or as required	- Place lozenge in the mouth between gum & cheek and allow to dissolve slowly (do not chew or swallow). - Move the lozenge from one side of the mouth to another until completely dissolved - A lozenge may take up to 30 mins to completely dissolve
 Mouth spray	SHORT ACTING Less than 5 minutes	Regular use or as required	- Has the fastest onset of action of all intermittent formulations so can also be used when cravings occur. - Hold the spray nozzle as close as possible to the mouth, avoiding the lips and press down on the top of the dispenser firmly to release one spray into the mouth. - Do NOT inhale or swallow while spraying but hold the spray in the mouth for 2-3 seconds before swallowing. - If cravings are still present after 1 minute, repeat once more.
 Patches	LONG ACTING 9-12 hours	Continuous application	- Apply patch to clean, dry, and hairless skin (do not shave as this may irritate skin), ensuring that it sticks well to the skin. - Remove the previous patch before applying a new one - Rotate the location of where the patch is applied to avoid skin irritation

Appendix 5: Clinically Significant Drug Interactions Associated with Smoking and Smoking Cessation

DRUG	NATURE OF INTERACTION WITH SMOKING Pharmacokinetic (PK) Pharmacodynamic (PD)	ACTION UPON CESSATION OF SMOKING	CLINICAL SIGNIFICANCE
Caffeine	PK: Increased clearance.	Advise to reduce caffeine by half.	High
Clozapine	PK: Increased clearance and decreased plasma concentrations.	Monitor trough plasma concentrations (if possible before stopping smoking and for two weeks after or sooner if adverse effects develop). Be alert for increased adverse effects. Reduce dose if clinically appropriate. Seek specialist advice from treating mental health practitioner.	High
Erlotinib	PK: Increased clearance and decreased plasma concentrations (around two-fold).	Reduce dose to initial starting dose if a patient stops smoking. Seek specialist advice. Nb. People who smoke should be encouraged to stop before therapy is initiated.	High
Irinotecan	PK: Increased clearance. Reduced exposure in people who smoke may lead to decreased haematological toxicity.	Seek specialist advice. Dosing should be closely monitored.	High
Theophylline	PK: Increased clearance and decreased half-life.	Monitor theophylline levels and reduce dose if clinically appropriate. Advise patient to monitor for signs of toxicity (e.g. palpitations, vomiting or nausea). Nb. It may take several weeks for enzyme induction to dissipate.	High
Chlorpromazine	PK: Decreased AUC and decreased plasma concentrations.	Be alert for increased adverse effects (e.g. dizziness, sedation, EPSE). Reduce dose if clinically appropriate.	Moderate
Insulin	Unclear: Possible decrease in insulin absorption secondary to peripheral vasoconstriction. Smoking may also increase insulin resistance.	Reduce dose if clinically appropriate. Advise patient to be alert for signs of hypoglycaemia and to test their BGLs more frequently.	Moderate
Methadone	Likely PK/PD: Nicotine affects the endogenous opioid system.	Be alert for signs of opioid toxicity. Reduce dose if clinically appropriate. Seek specialist advice. Nb. Methadone attenuates nicotine withdrawal.	Moderate
Olanzapine	PK: Increased clearance and decreased plasma concentrations.	Be alert for increased adverse effects (e.g. dizziness, sedation and hypotension). Reduce dose if clinically appropriate.	Moderate
Warfarin	PK: Increased clearance and decreased plasma concentrations.	Monitor INR closely. Reduce dose if clinically appropriate.	Moderate
Antiplatelet drugs (clopidogrel & prasugrel)	PK: Possible higher antiplatelet effect in people who smoke.	Seek specialist advice.	Low
Benzodiazepines	Likely PD: Central nervous system (CNS) stimulation by smoking. Nb. Results from pharmacokinetic studies are mixed.	Monitor for adverse effects (enhanced effect of benzodiazepines). Reduce dose if clinically appropriate.	Low
Beta Blockers	PD: Smoking opposes the beneficial effects of beta blockers on blood pressure and heart rate.	Monitor for adverse effects. Reduce dose if clinically appropriate.	Low
Haloperidol	PK: Decreased plasma concentrations.	Be alert for increased adverse effects (e.g. drowsiness, EPSE and hypotension). Reduce dose if clinically appropriate.	Low
Mirtazapine	PK: Decreased plasma concentrations.	Be alert for increased adverse effects (e.g. sedation). Reduce dose if clinically appropriate.	Low
Selective serotonin reuptake inhibitors (SSRIs)	PK: Decreased plasma concentrations. Nb. Best evidence for fluvoxamine, duloxetine and escitalopram.	Be alert for increased adverse effects (e.g. drowsiness and dizziness). Reduce dose if clinically appropriate.	Low
Tricyclic antidepressants	PK: Decreased plasma concentrations.	Be alert for increased adverse effects (e.g. sedation, dry mouth). Reduce dose if clinically appropriate.	Low

The most clinically significant interactions are provided here. For more information on any of the listed interactions or to search for other drug interactions, please refer to drug interactions references and literature at www.quit.org.au/psa-references